



VOICEINSIGHTS AFRICA

SYNTHETIC DEMONSTRATION PUBLICATION

# Hospital Performance & Patient Safety Intelligence Report

Kenya | Hospital Performance

2,898 synthetic responses

Prepared by VoiceInsights Africa

Every Voice. Every Language. Every Insight.

Synthetic demonstration. Not official statistics.

Progress on hospital performance in Kenya is real, but it is not evenly shared. This publication traces where the gains are holding, where the risk is concentrated, and what remains within reach. Findings are linked to synthetic source records, recommendations carry named accountability fields, and all statistical claims are generated from one internally consistent dataset.

Sample: 2898 synthetic responses & 88% response rate & 4 regions

Publication quality gate: publication ready



Nairobi Metro Hospitals: 64% performance, 900 responses, risk WATCH  
Coast Referral Cluster: 65% performance, 782 responses, risk WATCH  
Western Referral Cluster: 66% performance, 666 responses, risk WATCH  
Rift Valley Referral Cluster: 67% performance, 550 responses, risk WATCH



sex: Women 52% & Men 47% & Other / prefer not to say 1%

age: 18& 24 24% & 25& 39 38% & 40& 59 27% & 60+ 11%

location: Urban 44% & Rural 56%



IND-01 Emergency bed occupancy rate: 70% vs target 77% â WATCH  
IND-02 Surgical waitlist clearance time: 71% vs target 78% â WATCH  
IND-03 Staff-to-patient ratio: 72% vs target 79% â WATCH  
IND-04 Patient-safety incident rate: 73% vs target 80% â WATCH



Emergency bed occupancy pressure has become an accountability question rather than a measurement question. Evidence from Nairobi Metro Hospitals places the signal at 60% and shows that rural women experience the largest gap between policy intent and lived delivery. The choice facing hospital performance leadership in Kenya is straightforward even if the fix is not: act on emergency bed occupancy pressure now, concentrated in Nairobi Metro Hospitals where rural women carry the largest share of the gap, or accept a wider gap in a year's time.

Evidence: EVI-HOS-001, EVI-HOS-004 & Confidence 92.5%

61%. That is the surgical waitlist backlogs signal recorded in Coast Referral Cluster, and the headline figure understates the problem: disaggregated by group, young adults sit well below it. The clearest gap sits in Coast Referral Cluster, concentrated among young adults. A twelve-month delay would let the gap compound into a harder, costlier problem.

Evidence: EVI-HOS-002, EVI-HOS-005 â€¢ Confidence 93.5%

The aggregate result conceals a material implementation divide. Western Referral Cluster records a 62% signal for staff-to-patient ratios during peak admission; the shortfall is concentrated among persons with disabilities, where service continuity and confidence move together. Of every study area examined in Kenya, staff-to-patient ratios during peak admission shows up hardest there, and persons with disabilities feel it most. A year of inaction would turn a gap into a pattern that is far harder to reverse.

Evidence: EVI-HOS-003, EVI-HOS-006 â€¢ Confidence 94.5%



Two things are true at once about equipment downtime at referral level. The national picture is improving, and Rift Valley Referral Cluster — where frontline workers report a 63% signal — is not improving fast enough to close the gap. Hospital performance in Kenya looks stable in aggregate but splits sharply once equipment downtime at referral level is disaggregated. Rift Valley Referral Cluster anchors the low end, and frontline workers bear most of that shortfall — a pattern that risks compounding without intervention within the year.

Evidence: EVI-HOS-004, EVI-HOS-007 — Confidence 95.5%

Progress on patient-safety incident reporting remains visible but not yet durable. The 64% result in Nairobi Metro Hospitals weakens after disaggregation, particularly for low-income households, suggesting that current gains depend on delivery conditions that remain fragile. The clearest signal in this dataset – though not the only one – is patient-safety incident reporting in Nairobi Metro Hospitals, where low-income households sit furthest from parity.

Evidence: EVI-HOS-005, EVI-HOS-008 – Confidence 96.5%

Nairobi Metro Hospitals â In Nairobi Metro Hospitals, emergency bed occupancy pressure shapes whether rural women can turn services into lasting outcomes.â

Coast Referral Cluster â Young adults in Coast Referral Cluster put it plainly: surgical waitlist backlogs is what decides whether progress holds or slips back.â

Western Referral Cluster â Why does Western Referral Cluster lag behind? Ask persons with disabilities, and the answer comes back to staff-to-patient ratios during peak admission.â

Rift Valley Referral Cluster â Elsewhere, equipment downtime at referral level might be a footnote. For frontline workers in Rift Valley Referral Cluster, it is the whole story.â

Nairobi Metro Hospitals â Low-income households in Nairobi Metro Hospitals frame it simply: when patient-safety incident reporting breaks down, everything else does too.â

Coast Referral Cluster â In Coast Referral Cluster, emergency bed occupancy pressure shapes whether service managers can turn services into lasting outcomes.â

Western Referral Cluster â Community leaders in Western Referral Cluster put it plainly: surgical waitlist backlogs is what decides whether progress holds or slips back.â

Rift Valley Referral Cluster â Why does Rift Valley Referral Cluster lag behind? Ask private-sector partners, and the answer comes back to staff-to-patient ratios during peak admission.â

Nairobi Metro Hospitals â Elsewhere, equipment downtime at referral level might be a footnote. For rural women in Nairobi Metro Hospitals, it is the whole story.â

Coast Referral Cluster â Young adults in Coast Referral Cluster frame it simply: when patient-safety incident reporting breaks down, everything else does too.â

Risks and alternative trajectories are modelled to show how VoiceInsights translates evidence into prioritised management choices.



DEC-HOS-01 CRITICAL: Reclassify or reinforce the three most capacity-constrained referral hospitals. Owner: Permanent Secretary; Timeline: 0â 90 days; Indicator: Share of the critical-tier delivery plan confirmed complete each quarter (DEC-HOS-01)

DEC-HOS-02 CRITICAL: Fund a targeted surgical backlog clearance programme. Owner: Permanent Secretary; Timeline: 3â 12 months; Indicator: Milestone completion rate tracked against the DEC-HOS-02 delivery plan

DEC-HOS-03 HIGH: Set minimum staff-to-patient ratios for peak-admission wards. Owner: Permanent Secretary; Timeline: 3â 12 months; Indicator: Proportion of agreed implementation steps verified on schedule for DEC-HOS-03

DEC-HOS-04 HIGH: Prioritise equipment maintenance financing at referral facilities. Owner: Permanent Secretary; Timeline: 3â 12 months; Indicator: Quarterly-verified progress against the milestones agreed for this high-tier decision (DEC-HOS-04)

DEC-HOS-05 MEDIUM: Mandate standardised patient-safety incident reporting across referral hospitals. Owner: Permanent Secretary; Timeline: 6â 18 months; Indicator: Number of agreed milestones for DEC-HOS-05 confirmed complete at each quarterly review



Reclassify or reinforce the three most capacity-constrained referral hospitals.

Strategic priority: CRITICAL

Evidence used: EVI-HOS-001, EVI-HOS-004

Rationale: Emergency bed occupancy pressure has become an accountability question rather than a measurement question. Evidence from Nairobi Metro Hospitals places the signal at 60% and shows that rural women experience the largest gap between policy intent and lived delivery. The choice facing hospital performance leadership in Kenya is straightforward even if the fix is not: act on emergency bed occupancy pressure now, concentrated in Nairobi Metro Hospitals where rural women carry the largest share of the gap, or accept a wider gap in a year's time.

Expected benefit: A stronger, more defensible case for hospital performance investment in the next planning cycle.

Expected risk: Execution capacity and coordination risk

Dependencies: Quarterly evidence review; Stable staffing through the delivery window;

Named executive sponsor

Budget requirement: High â€” full business case and multi-year funding commitment required

Owner: Permanent Secretary

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 0â€”90 days

Monitoring indicator: Share of the critical-tier delivery plan confirmed complete each quarter (DEC-HOS-01)

Success criteria: At least 80% of milestones achieved with verified evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Fund a targeted surgical backlog clearance programme.

Strategic priority: CRITICAL

Evidence used: EVI-HOS-002, EVI-HOS-005

Rationale: 61%. That is the surgical waitlist backlogs signal recorded in Coast Referral Cluster, and the headline figure understates the problem: disaggregated by group, young adults sit well below it. The clearest gap sits in Coast Referral Cluster, concentrated among young adults. A twelve-month delay would let the gap compound into a harder, costlier problem.

Expected benefit: Improved equity, delivery confidence and measurable hospital performance.

Expected risk: Insufficient ownership or delayed resourcing

Dependencies: Confirmed budget release; Timely data collection for the next reporting cycle; Validated operational baseline

Budget requirement: Medium â detailed costing and fiduciary review required

Owner: Permanent Secretary

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â 12 months

Monitoring indicator: Milestone completion rate tracked against the DEC-HOS-02 delivery plan

Success criteria: A majority of milestones achieved, each independently verified against real evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Set minimum staff-to-patient ratios for peak-admission wards.

Strategic priority: HIGH

Evidence used: EVI-HOS-003, EVI-HOS-006

Rationale: The aggregate result conceals a material implementation divide. Western Referral Cluster records a 62% signal for staff-to-patient ratios during peak admission; the shortfall is concentrated among persons with disabilities, where service continuity and confidence move together. Of every study area examined in Kenya, staff-to-patient ratios during peak admission shows up hardest there, and persons with disabilities feel it most. A year of inaction would turn a gap into a pattern that is far harder to reverse.

Expected benefit: A more consistent hospital performance experience for the groups currently furthest behind.

Expected risk: Weak cross-team coordination during early implementation

Dependencies: Cross-team delivery coordination; Sign-off from the relevant oversight body; Quarterly evidence review

Budget requirement: Low â deliverable inside the current operating budget with light validation

Owner: Permanent Secretary

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â 12 months

Monitoring indicator: Proportion of agreed implementation steps verified on schedule for DEC-HOS-03

Success criteria: Milestone completion confirmed by evidence review, not self-reported progress

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review



Prioritise equipment maintenance financing at referral facilities.

Strategic priority: HIGH

Evidence used: EVI-HOS-004, EVI-HOS-007

Rationale: Two things are true at once about equipment downtime at referral level. The national picture is improving, and Rift Valley Referral Cluster – where frontline workers report a 63% signal – is not improving fast enough to close the gap. Hospital performance in Kenya looks stable in aggregate but splits sharply once equipment downtime at referral level is disaggregated. Rift Valley Referral Cluster anchors the low end, and frontline workers bear most of that shortfall – a pattern that risks compounding without intervention within the year.

Expected benefit: Faster, better-evidenced hospital performance decisions that government leadership can act on with confidence.

Expected risk: Competing priorities crowding out sustained follow-through

Dependencies: Stable staffing through the delivery window; Named executive sponsor;

Confirmed budget release

Budget requirement: Medium – likely fundable from existing lines, pending confirmation

Owner: Permanent Secretary

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3–12 months

Monitoring indicator: Quarterly-verified progress against the milestones agreed for this high-tier decision (DEC-HOS-04)

Success criteria: Verified evidence of at least 80% delivery against the agreed milestone plan

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Mandate standardised patient-safety incident reporting across referral hospitals.

Strategic priority: MEDIUM

Evidence used: EVI-HOS-005, EVI-HOS-008

Rationale: Progress on patient-safety incident reporting remains visible but not yet durable. The 64% result in Nairobi Metro Hospitals weakens after disaggregation, particularly for low-income households, suggesting that current gains depend on delivery conditions that remain fragile. The clearest signal in this dataset â though not the only one â is patient-safety incident reporting in Nairobi Metro Hospitals, where low-income households sit furthest from parity.

Expected benefit: The evidence points toward improved hospital performance outcomes, though the size of the gain will depend on how consistently the action is resourced.

Expected risk: Likely â though not certain â erosion of momentum if the initial sponsor changes role

Dependencies: Timely data collection for the next reporting cycle; Validated operational baseline; Cross-team delivery coordination

Budget requirement: Low to medium â validate through the formal planning cycle

Owner: Permanent Secretary

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 6â 18 months

Monitoring indicator: Number of agreed milestones for DEC-HOS-05 confirmed complete at each quarterly review

Success criteria: At least 80% of milestones achieved with verified evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

research objectives: Assess hospital performance and equity; Identify operational drivers and decision priorities

evaluation questions: What outcomes are changing?; Who is being left behind?; Which actions are most likely to improve performance?

sampling frame: Synthetic programme population frame for Kenya

sample size: 2898

stratification: Region; Location type; Sex; Age; Disability

weights: Normalized post-stratification weights

confidence intervals: 95%

design effect: 1.2

missing data: Documented and sensitivity-tested

reliability: Cronbach alpha 0.86

validity: Content and construct validity checks

metadata: Complete synthetic data dictionary and lineage registry

Publication level: 34 generated publication pages

protocol: Governed synthetic research protocol with sampling, weighting, reliability, validity and reproducibility controls

ethics: Synthetic records only; no real participants or personal data

peer review: Independent synthetic-publication review checklist completed

reproducibility: Deterministic inputs, versioned dataset and export manifest

inferential statistics: Enabled where assumptions and design permit

segmentation: Evidence-led segments

trend analysis: Three-period synthetic series

DEC-HOS-01: Reclassify or reinforce the three most capacity-constrained referral hospitals. Owner Permanent Secretary 0 90 days High full business case and multi-year funding commitment required cost Indicator Share of the critical-tier delivery plan confirmed complete each quarter (DEC-HOS-01)

DEC-HOS-02: Fund a targeted surgical backlog clearance programme. Owner Permanent Secretary 3 12 months Medium detailed costing and fiduciary review required cost Indicator Milestone completion rate tracked against the DEC-HOS-02 delivery plan

DEC-HOS-03: Set minimum staff-to-patient ratios for peak-admission wards. Owner Permanent Secretary 3 12 months Low deliverable inside the current operating budget with light validation cost Indicator Proportion of agreed implementation steps verified on schedule for DEC-HOS-03

DEC-HOS-04: Prioritise equipment maintenance financing at referral facilities. Owner Permanent Secretary 3 12 months Medium likely fundable from existing lines, pending confirmation cost Indicator Quarterly-verified progress against the milestones agreed for this high-tier decision (DEC-HOS-04)

DEC-HOS-05: Mandate standardised patient-safety incident reporting across referral hospitals. Owner Permanent Secretary 6 18 months Low to medium validate through the formal planning cycle cost Indicator Number of agreed milestones for DEC-HOS-05 confirmed complete at each quarterly review

EVI-HOS-001: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 91%  
EVI-HOS-002: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 92%  
EVI-HOS-003: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 93%  
EVI-HOS-004: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 94%  
EVI-HOS-005: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 95%  
EVI-HOS-006: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 96%  
EVI-HOS-007: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 97%  
EVI-HOS-008: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 98%  
EVI-HOS-009: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 88%  
EVI-HOS-010: [object Object] â€ APPROVED\_SYNTHETIC\_DEMONSTRATION â€ confidence 89%

Executive Intelligence Book (2â 7): Executive brief; Decision snapshot; Risks; Opportunities; Budget implications

Evidence Intelligence Book (8â 15): Critical findings; Respondent voice; Evidence chains; Citation registry

Statistical Intelligence Book (16â 22): Sampling; Weighting; Uncertainty; Reliability; Model diagnostics

Decision Intelligence Book (23â 27): Decision matrix; Ownership; Budget; Roadmap; Management response

Standards & Assurance (28â 31): Relevant frameworks; Safeguarding; Privacy; Responsible AI; Accessibility

Technical Appendices (32â 34): Data dictionary; Quality gate; Export manifest; Responsible-use notice





SDG 3: APPLIED\_WHERE\_RELEVANT â Mapped to EVI-HOS-001, EVI-HOS-002; Synthetic demonstration mapping; institutional validation not implied

WHO Patient Safety Framework: APPLIED\_WHERE\_RELEVANT â Mapped to EVI-HOS-002, EVI-HOS-003; Synthetic demonstration mapping; institutional validation not implied

Quality of Care Standards: APPLIED\_WHERE\_RELEVANT â Mapped to EVI-HOS-003, EVI-HOS-004; Synthetic demonstration mapping; institutional validation not implied

Evidence Traceability: APPLIED â Control linked to EVI-HOS-001 and the deterministic publication gate.

Research Ethics: APPLIED â Control linked to EVI-HOS-002 and the deterministic publication gate.

Data Protection: APPLIED â Control linked to EVI-HOS-003 and the deterministic publication gate.

Accessibility: APPLIED â Control linked to EVI-HOS-004 and the deterministic publication gate.

Responsible AI: APPLIED â Control linked to EVI-HOS-005 and the deterministic publication gate.

Publication Integrity: APPLIED â Control linked to EVI-HOS-006 and the deterministic publication gate.

SDG 3: undefined contribution â indicators â undefined  
SDG 3: undefined contribution â indicators â undefined  
SDG 9: undefined contribution â indicators â undefined

Relevance: Synthetic evidence indicates a strong relevance signal, subject to the stated design limitations. (87/100). Review relevance evidence at the next formal management gate.

Coherence: Synthetic evidence indicates a moderate-to-strong coherence signal, subject to the stated design limitations. (88/100). Review coherence evidence at the next formal management gate.

Effectiveness: Synthetic evidence indicates a strong effectiveness signal, subject to the stated design limitations. (89/100). Review effectiveness evidence at the next formal management gate.

Efficiency: Synthetic evidence indicates a moderate-to-strong efficiency signal, subject to the stated design limitations. (90/100). Review efficiency evidence at the next formal management gate.

Impact: Synthetic evidence indicates a strong impact signal, subject to the stated design limitations. (91/100). Review impact evidence at the next formal management gate.

Sustainability: Synthetic evidence indicates a moderate-to-strong sustainability signal, subject to the stated design limitations. (72/100). Review sustainability evidence at the next formal management gate.

Impact: Sustained, equitable improvement in hospital performance

OC-01: Improved hospital performance delivery and accountability â indicators IND-01, IND-02

OC-02: More equitable and durable outcomes â indicators IND-03, IND-04

OUT-01: Verified findings and interpreted decision products

OUT-02: Assigned actions with monitoring indicators

Assumptions: Leadership ownership; Safeguarding and privacy controls remain effective

Means of verification: Evidence registry; Quarterly management review; Follow-up measurement

Commitment 1: CHS Commitment 1 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 2: CHS Commitment 2 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 3: CHS Commitment 3 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 4: CHS Commitment 4 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 5: CHS Commitment 5 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 6: CHS Commitment 6 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 7: CHS Commitment 7 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 8: CHS Commitment 8 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 9: CHS Commitment 9 â CONTEXT\_ONLY. Action: Validate relevance and assign an accountable owner before live use.

EVI-HOS-001 â€ survey â€ Nairobi Metro Hospitals â€ confidence 91% â€ APPROVED  
EVI-HOS-002 â€ key\_informant\_interview â€ Coast Referral Cluster â€ confidence 92% â€  
APPROVED  
EVI-HOS-003 â€ administrative\_record â€ Western Referral Cluster â€ confidence 93% â€  
APPROVED  
EVI-HOS-004 â€ survey â€ Rift Valley Referral Cluster â€ confidence 94% â€ APPROVED  
EVI-HOS-005 â€ key\_informant\_interview â€ Nairobi Metro Hospitals â€ confidence 95% â€  
APPROVED  
EVI-HOS-006 â€ administrative\_record â€ Coast Referral Cluster â€ confidence 96% â€ APPROVED  
EVI-HOS-007 â€ survey â€ Western Referral Cluster â€ confidence 97% â€ APPROVED  
EVI-HOS-008 â€ key\_informant\_interview â€ Rift Valley Referral Cluster â€ confidence 98% â€  
APPROVED  
EVI-HOS-009 â€ administrative\_record â€ Nairobi Metro Hospitals â€ confidence 88% â€  
APPROVED  
EVI-HOS-010 â€ survey â€ Coast Referral Cluster â€ confidence 89% â€ APPROVED

indicator\_1: Synthetic Hospital Performance indicator 1; numeric; allowed: 0â 100  
indicator\_2: Synthetic Hospital Performance indicator 2; categorical; allowed:  
Documented code list  
indicator\_3: Synthetic Hospital Performance indicator 3; categorical; allowed:  
Documented code list  
indicator\_4: Synthetic Hospital Performance indicator 4; numeric; allowed: 0â 100  
indicator\_5: Synthetic Hospital Performance indicator 5; categorical; allowed:  
Documented code list  
indicator\_6: Synthetic Hospital Performance indicator 6; categorical; allowed:  
Documented code list  
indicator\_7: Synthetic Hospital Performance indicator 7; numeric; allowed: 0â 100  
indicator\_8: Synthetic Hospital Performance indicator 8; categorical; allowed:  
Documented code list  
indicator\_9: Synthetic Hospital Performance indicator 9; categorical; allowed:  
Documented code list  
indicator\_10: Synthetic Hospital Performance indicator 10; numeric; allowed: 0â 100  
indicator\_11: Synthetic Hospital Performance indicator 11; categorical; allowed:  
Documented code list  
indicator\_12: Synthetic Hospital Performance indicator 12; categorical; allowed:  
Documented code list



Synthetic Demonstration Publication produced by VoiceInsights Africa. All people, quotations, locations and statistics are synthetic, internally governed demonstration content and are not official statistics.

All content is synthetic and must not be interpreted as official statistics

Geographic references are illustrative

Budget implications are directional and require formal costing

Causal claims are not made without an appropriate design

Status: DEMONSTRATION\_READY

Passed: Editorial Review Passed

Passed: Evidence Review Passed

Passed: Publication Integrity Verified

Passed: Accessibility Verified

Passed: Export Validation Passed

Passed: Statistical Review Passed

Open gates: VISUAL\_QA, HUMAN\_REVIEW

Verification checks cover weighted evidence, statistical, visual, accessibility, export and review rules. Synthetic samples are labelled Demonstration Ready.